

SPARTAN COACHING

DISCIPLINE | EMPATHY | STRATEGY

PHYSICIAN ENGAGEMENT STRATEGY

Building Medical Referral Relationships Through Clinical Credibility

A complete framework for earning physician trust, speaking the clinical language, structuring CME partnerships, and converting skeptics into long-term referral champions.

Physician Engagement Strategy

Earning the Trust of the Most Difficult Referral Source in Healthcare

UNDERSTANDING THE PHYSICIAN MINDSET

Physicians are trained to be skeptical. They are evidence-driven, time-constrained, and have been pitched by pharmaceutical reps, device companies, and other service providers for their entire careers. They have excellent radar for when someone is wasting their time or talking to them without genuine clinical understanding.

The most common mistake hospice sales reps make with physicians is leading with features and benefits. Physicians do not care about your hospice's response times, your star ratings, or your service coverage area in the first conversation. They care about whether you understand their patients, their challenges, and their clinical reasoning. Earn that first, and the operational details follow naturally.

THE FIVE PHYSICIAN ARCHETYPES

1. The Paternalistic Physician (Most Common)

This physician has a deep emotional attachment to their patients and views hospice conversations as admissions of defeat. They delay referrals because accepting a terminal prognosis feels like abandonment. Approach with respect for their commitment and reframe hospice as a continuation of their care, not an ending.

2. The Busy Practitioner

This physician simply does not have time to think carefully about their patients with serious illness. They are managing a panel of 2,000 people and the hospice conversation falls to the bottom of the priority stack. Approach by making the process as frictionless as possible and offering to handle the family conversation on their behalf.

3. The Evidence Follower

This physician wants clinical data before making any referral decision. They will respond to published research on hospice outcomes, symptom management effectiveness, and quality of life metrics. Come prepared with peer-reviewed evidence and speak in clinical terms.

4. The Champion (Your Goal)

This physician has had a transformative experience with hospice, either personally or clinically, and has become an advocate. They refer early and often. Maintain these relationships with consistency and excellent follow-through. Protect them carefully because their loyalty is your biggest competitive advantage.

5. The Burned Skeptic

This physician had a bad experience with hospice previously. A late admission, poor clinical support, or communication failure created permanent skepticism. Identify the specific incident if possible, acknowledge it directly, and demonstrate through actions rather than words that your team is different.

THE CLINICAL CONVERSATION FRAMEWORK

Opening with Clinical Relevance

"Dr. [Name], I appreciate the few minutes. I work with physicians managing patients with advanced heart failure and COPD, and I know that the conversation about shifting goals of care is one of the hardest conversations in medicine. I wanted to ask you directly: in your experience, at what point do your patients typically begin expressing that they want to prioritize comfort over curative intervention?"

Three Clinical Talking Points That Resonate

1 The Outcome Data Point

"Studies in JAMA and NEJM have consistently shown that patients with advanced cancer or CHF who enroll in hospice earlier experience lower symptom burden, fewer hospitalizations in their final months, and in some cases live longer than similar patients who continue aggressive treatment. The evidence is genuinely compelling."

2 The Care Continuation Point

"Many physicians worry that referring to hospice means losing their relationship with the patient. In practice, most of our physicians stay actively involved. We send clinical updates at every meaningful change and we actively seek their input on symptom management decisions. Hospice extends your care — it does not replace it."

3 The Administrative Relief Point

"I know the certification documentation can be time-intensive. We have a process that minimizes what you need to do on your end. We handle the clinical narrative, we coordinate with your office for signature, and we send a clear summary of what you need to certify and why. Most physicians tell me it takes less than three minutes of their time."

CME PARTNERSHIP OPPORTUNITIES

Educational programming is one of the highest-value relationship builders with physicians because it positions your hospice as a clinical resource rather than a sales vendor. The following formats work well in physician settings:

- **Grand Rounds:** 30 to 45 minute presentation on a clinically relevant topic such as symptom management in advanced heart failure or the evidence base for early hospice enrollment. Best for hospital-based physicians.
- **Lunch and Learn:** 15 to 20 minute educational session at the practice. Focus on eligibility criteria for the specialty's primary diagnoses. Bring food. Keep slides to a minimum.
- **Case Conference:** Monthly or quarterly review of complex patients in the physician's panel. Your hospice medical director participates alongside the attending. Builds the deepest clinical trust.
- **Written Resource:** A one-page clinical reference card tailored to the specialty's primary diagnoses, featuring eligibility criteria, clinical indicators, and your contact information.

SAMPLE QUARTERLY BUSINESS REVIEW AGENDA

Conduct a brief business review with your highest-volume physician partners every 90 days. Keep it to 20 minutes or less. The agenda signals that you take the partnership seriously and are tracking outcomes.

Agenda: 20-Minute QBR with a High-Volume Physician Partner

1 Outcomes Update (5 min)

Share aggregate data on patients referred over the prior quarter: average length of stay, family satisfaction summary, any notable care moments you can share without violating HIPAA.

2 Process Review (5 min)

Ask if the referral process is working smoothly. Address any documentation friction, response time concerns, or communication gaps.

3 Clinical Education (5 min)

Share one clinical update, case study, or data point that is relevant to the cases they are seeing most often.

4 Next Quarter Planning (5 min)

Identify any specific patients or case types where you can provide more support. Agree on next touchpoint.

CONVERSION TIMELINE EXPECTATIONS

Physician relationship development is a long-cycle process. Do not expect referrals within the first two or three visits. Below are realistic timeline expectations based on Spartan Method application:

- Months 1 to 2: Introduction and education phase. You are planting seeds and establishing credibility. No referrals expected.
- Month 3: First informal referral inquiry or request for more information. This is a positive signal.
- Months 4 to 5: First patient referral, usually a patient the physician has been thinking about for some time. Deliver an exceptional experience.
- Month 6 onward: Regular referral pattern begins if you have delivered on the first one. This is when consistency of visits and follow-through compounds into a champion relationship.

THE PHYSICIAN RELATIONSHIP RULE

Never waste a physician's time. Not their physical time in a meeting, and not their cognitive time with a disorganized pitch. Be brief, be clinical, and be prepared. Every second you spend on company boilerplate is a second you could spend asking a question that builds trust. Physicians reward those who respect their expertise by treating them as clinical peers, not as sales targets.